

COMMITTEE ON HEALTH AND HUMAN SERVICES
SENATE AMENDMENTS TO H.B. 2195
(Reference to House engrossed bill)

Amendment instruction key:

[GREEN UNDERLINING IN BRACKETS] indicates text added to statute or previously enacted session law.

[Green underlining in brackets] indicates text added to new session law or text restoring existing law.

[GREEN STRIKEOUT IN BRACKETS] indicates new text removed from statute or previously enacted session law.

[Green strikeout in brackets] indicates text removed from existing statute, previously enacted session law or new session law.

<<Green carets>> indicate a section added to the bill.

<<Green strikeout in carets>> indicates a section removed from the bill.

1 The bill as proposed to be amended is reprinted as follows:

2 Section 1. Section 36-406, Arizona Revised Statutes, is amended to
3 read:

4 36-406. Powers and duties of the department; access to
5 personnel records

6 A. In addition to its other powers and duties:

7 1. The department shall:

8 (a) Administer and enforce this chapter and the rules, regulations
9 and standards adopted pursuant ~~thereto~~ TO THIS CHAPTER.

10 (b) Review, and may approve, plans and specifications for
11 construction or modification OF or additions to health care institutions
12 regulated by this chapter.

13 (c) EXCEPT AS PROVIDED IN SUBSECTION B OF THIS SECTION, have access
14 to books, records, accounts and any other information of any health care
15 institution THAT ARE reasonably necessary for the purposes of this
16 chapter.

17 (d) Require as a condition of licensure that nursing care
18 institutions and assisted living facilities make vaccinations for
19 influenza and pneumonia available to residents ~~on-site~~ ON-SITE on a yearly
20 basis. The department shall prescribe the manner by which the NURSING
21 CARE institutions and ASSISTED LIVING facilities ~~shall~~ MUST document
22 compliance with this subdivision, including documenting residents who
23 refuse to be immunized. The department shall not impose a violation on a
24 licensee for not making a vaccination available if there is a shortage of
25 that vaccination in this state as determined by the director.

26 2. The department may:

27 (a) Make or cause to be made inspections consistent with standard
28 medical practice of every part of the premises of health care institutions

1 ~~which~~ THAT are subject to ~~the provisions of~~ this chapter as well as those
2 ~~which~~ THAT apply for or hold a license required by this chapter.

3 (b) Make studies and investigations of conditions and problems in
4 health care institutions, or any class or subclass thereof, as they relate
5 to compliance with this chapter and rules, regulations and standards
6 adopted pursuant ~~thereto~~ TO THIS CHAPTER.

7 (c) Develop manuals and guides relating to any of the several
8 aspects of physical facilities and operations of health care institutions
9 or any class or subclass thereof for distribution to the governing
10 authorities of health care institutions and to the general public.

11 B. THE DEPARTMENT'S ACCESS TO THE PERSONNEL RECORDS OF A NURSING
12 CARE INSTITUTION IS LIMITED TO THE FOLLOWING INFORMATION FOR EACH
13 PERSONNEL MEMBER, EMPLOYEE OR VOLUNTEER:

14 1. THE INDIVIDUAL'S NAME, DATE OF BIRTH AND CONTACT TELEPHONE
15 NUMBER.

16 2. THE INDIVIDUAL'S STARTING DATE OF EMPLOYMENT OR VOLUNTEER
17 SERVICE AND, IF APPLICABLE, THE ENDING DATE.

18 3. DOCUMENTATION OF ALL OF THE FOLLOWING AS PERTAINING TO THE
19 INDIVIDUAL:

20 (a) QUALIFICATIONS, INCLUDING SKILLS AND KNOWLEDGE APPLICABLE TO
21 THE INDIVIDUAL'S JOB DUTIES.

22 (b) EDUCATION AND EXPERIENCE APPLICABLE TO THE INDIVIDUAL'S JOB
23 DUTIES.

24 (c) COMPLIANCE WITH THE REQUIREMENTS OF SECTION 36-411.

25 (d) ORIENTATION AND IN-SERVICE EDUCATION AS REQUIRED BY POLICIES
26 AND PROCEDURES.

27 (e) LENSURE OR CERTIFICATION, IF THE INDIVIDUAL IS REQUIRED TO BE
28 LICENSED OR CERTIFIED PURSUANT TO THIS ARTICLE OR POLICIES AND PROCEDURES.

29 (f) IF THE INDIVIDUAL IS A BEHAVIORAL HEALTH TECHNICIAN, CLINICAL
30 OVERSIGHT AS REQUIRED BY RULE.

31 (g) CARDIOPULMONARY RESUSCITATION TRAINING, IF REQUIRED FOR THE
32 INDIVIDUAL BY RULE.

33 (h) FIRST AID TRAINING, IF REQUIRED FOR THE INDIVIDUAL PURSUANT TO
34 THIS ARTICLE OR POLICIES AND PROCEDURES.

35 (i) EVIDENCE OF FREEDOM FROM INFECTIOUS TUBERCULOSIS, IF REQUIRED
36 FOR THE INDIVIDUAL BY RULE.

37 (j) IF THE INDIVIDUAL IS A NUTRITION AND FEEDING ASSISTANT, BOTH:

38 (i) COMPLETION OF THE NUTRITION AND FEEDING ASSISTANT TRAINING
39 COURSE AS REQUIRED BY RULE.

40 (ii) A NURSE'S OBSERVATIONS AS REQUIRED BY RULE.

1 Sec. 2. Title 36, chapter 4, article 7, Arizona Revised Statutes,
2 is amended by adding ~~[section]~~ [sections] 36-447.03 [and 36-447.04], to
3 read:

4 36-447.03. Surveys; statement of deficiencies; initiation of
5 complaint investigations; time frames; record
6 retention

7 A. [BEGINNING JULY 1, 2027.] THE DEPARTMENT SHALL ISSUE ANY
8 STATEMENT OF DEFICIENCIES TO A NURSING CARE INSTITUTION WITHIN TEN
9 BUSINESS DAYS AFTER COMPLETING THE STATE SURVEY. [NOTWITHSTANDING THE
10 DATE THAT THE STATEMENT OF DEFICIENCIES IS ISSUED, THE NURSING CARE
11 INSTITUTION SHALL COMPLY WITH THE STATEMENT OF DEFICIENCIES AND SUBMIT A
12 PLAN OF CORRECTION TO THE DEPARTMENT.]

13 B. THE DEPARTMENT MAY NOT INITIATE A COMPLAINT INVESTIGATION
14 RELATING TO A NURSING CARE INSTITUTION MORE THAN TWELVE MONTHS AFTER THE
15 ALLEGED BASIS OF THE INCIDENT OCCURRED UNLESS THE COMPLAINT INVOLVES
16 ALLEGATIONS OF CRIMINAL ABUSE OR NEGLECT ~~[AND]~~ [OR] A FAILURE BY A PERSON
17 WHO HAS A DUTY TO REPORT THE POSSIBLE ABUSE OR NEGLECT PURSUANT TO SECTION
18 13-3620 OR 46-454. IF RECORDS THAT ARE MAINTAINED PURSUANT TO ARTICLE 4
19 OF THIS CHAPTER HAVE BEEN LAWFULLY DESTROYED PURSUANT TO RETENTION
20 REQUIREMENTS, THE DEPARTMENT MAY NOT CITE THE LICENSEE FOR FAILURE TO
21 PRODUCE THE RECORDS FOR AN INVESTIGATION INITIATED MORE THAN TWELVE MONTHS
22 AFTER THE ALLEGED BASIS OF THE INCIDENT OCCURRED.

23 C. THE DEPARTMENT SHALL INITIATE COMPLAINT INVESTIGATIONS RELATING
24 TO NURSING CARE INSTITUTIONS CONSISTENT WITH THE CENTERS FOR MEDICARE AND
25 MEDICAID SERVICES TIME FRAMES BASED ON THE TRIAGED SEVERITY LEVEL OF EACH
26 COMPLAINT.

27 36-447.04. Off-site preliminary reviews; compliance
28 inspections; complaint investigations

29 [THE DEPARTMENT MAY CONDUCT AN OFF-SITE PRELIMINARY REVIEW OF A
30 COMPLAINT OR SELF-REPORT USING WRITTEN OR VERBAL COMMUNICATIONS OR
31 DOCUMENTATION FROM THE LICENSEE TO DETERMINE WHETHER AN ON-SITE COMPLIANCE
32 INSPECTION OR COMPLAINT INVESTIGATION IS NECESSARY. IF THE REVIEW
33 CONFIRMS THAT THE LICENSEE HAS IMPLEMENTED SYSTEMS AND PROCESSES THAT MAKE
34 FURTHER VIOLATIONS UNLIKELY AND THE LICENSEE PROVIDES SUFFICIENT EVIDENCE
35 OF COMPLIANCE, THE DEPARTMENT SHALL CLOSE THE COMPLAINT AND NO FURTHER
36 INVESTIGATION IS REQUIRED.]

37 Sec. 3. Section 36-2402, Arizona Revised Statutes, is amended to
38 read:

39 36-2402. Quality assurance activities; sharing of quality
40 assurance information; immunity; confidentiality

41 A. State health care providers, hospitals and outpatient surgical
42 centers shall, and other health care entities may, conduct quality
43 assurance activities.

44 B. A health care entity may share quality assurance information
45 with appropriate state licensing or certifying agencies and with licensed
46 health care providers who are the subject of quality assurance
47 activities. A hospital may share quality assurance information with other

1 health care entities only with the approval of the hospital's medical
2 executive committee or an equivalent committee.

3 C. A health care entity may share quality assurance information
4 with other health care entities only for the purpose of conducting quality
5 assurance activities.

6 D. A health care entity or person that provides or receives
7 information, that participates, takes any action or makes any decision or
8 recommendation in the course of quality assurance activities or that
9 furnishes any records, information or assistance to a health care entity
10 for or in the course of quality assurance activities is not subject to
11 liability for civil damages or any legal action in consequence of such
12 action except as provided in section 36-445.02.

13 E. Quality assurance activities conducted by state, county or local
14 medical, pharmacy and dental associations and societies on behalf of a
15 health care entity are immune from civil liability to the same degree as
16 the facility for which the review activities are conducted.

17 F. Health care entities may jointly conduct quality assurance
18 activities.

19 G. This section does not relieve any health care entity from
20 liability arising from the treatment of a patient or from negligent
21 credentialing decisions.

22 H. LONG-TERM CARE INCIDENT REPORTS:

23 1. ARE A RISK MANAGEMENT TOOL TO BE USED SOLELY FOR INTERNAL
24 INVESTIGATIONS AND QUALITY IMPROVEMENT EFFORTS.

25 2. ARE CONFIDENTIAL RECORDS OF AN UNPLANNED EVENT OR OCCURRENCE
26 THAT IS INCONSISTENT WITH ROUTINE CARE AND THAT MAY IMPACT A RESIDENT'S
27 SAFETY, HEALTH OR WELL-BEING.

28 3. ARE USED TO SUPPORT FOLLOW-UP ACTIONS AND IDENTIFY OPPORTUNITIES
29 FOR IMPROVEMENT AND MANAGE RISK.

30 4. ARE MAINTAINED SEPARATELY FROM THE RESIDENT'S MEDICAL RECORD.

31 Enroll and engross to conform

32 Amend title to conform

2195HHS.docx

03/04/2026

05:03 PM

C: MH

130AJCFCG